pharmaceutical firm Macleods has a licensing agreement with TB Alliance to supply Pretomanid, it took India nearly two years to finally approve the rollout of BPaL last month. Sources blame it on red-tapism, while the government claims it had been waiting for the results of domestic trials—which, ironically, are still awaited.

Also, despite the several advantages of the new regimen, experts remain cautious. For starters, Linezolid has had many patient-reported side-effects such as headache, nausea, liver issues and, in some cases, neuropathy, which impacts their ability to walk. "It is perhaps not as toxic as some of the injectable drugs once used for TB treatment, but Linezolid is not completely free of side-effects," says Chapal Mehra, a public health expert and director of Survivors Against TB. There are also concerns about doctor-reported Bedaquiline resistance in the country. It has been extremely effective in treating DR-TB so far, but if the bacteria were to develop widespread resistance to it, treatment would become extremely challenging. To ensure safe use of this drug, experts say the government needs to regulate access to it and ensure full patient compliance. More than anything, however, it is the problematic and erratic supply of drugs even for DS-TB that is a cause for major concern.

INDIA'S TB BURDEN

2.55 MN
TB cases notified in India in 2023

27%
India's share in global TB cases

331,000
TB deaths in India in 2022

25%
India's share in global TB deaths

63,939
Multidrug-resistant TB cases notified in 2023

Source: WHO Global TB Report 2023 & India TB Report 2024

The Supply Pangs

Procurement of TB drugs is centralised under the National TB Elimination Programme and follows a complex chain of responsibility. Typically, the Central Medical Services Society (CMSS), tasked with procuring drugs for the health ministry, initiates the process by floating tenders and selecting suppliers. Once the central procurement is complete, these vital medications are dispatched to State Drug Stores or directly to districts, guided by quarterly reports submitted through the Nikshay Aushadhi portal. From the district level, they are transferred to Tuberculosis Units (TUs), which then supply them to Peripheral Health Institutes (PHIs). Finally, the PHIs issue the drugs to patients. This intricate supply chain usually takes about 15-20 days to replenish drug stocks.

Historically, pharmaceutical firm Lupin had been supplying DS-TB drugs to the CMSS. This changed in September 2023, says a source in the agency, as fresh tenders were floated to procure 4FDC and 3FDC. Days later, on October 1, the Centre in a press release claimed it had adequate stocks of DS-TB drugs even as the data shared included stocks still in the pipeline. The source cited above claims the two companies that won the bid could not fulfil the orders in a timely manner, leading to a supply crunch. As a result, this year, the government floated an 'emergency' tender for the supply of drugs—first on March 8 and then again on March 22. Meanwhile, on March 18, the Union government had written to all the states, asking them to procure DS-TB drugs for three months using the National Health Mission (NHM) funds. "Central procurement of [the drugs] is at an advanced stage. However, supplies may get delayed due to unforeseen and extraneous circumstances," the letter by Dr Rajendra Joshi, deputy director general, Central TB Division, had stated.

States were also given the option to reimburse patients who bought the medication on their own. However, not just the patients, even states don't have the experience or the bargaining power to negotiate prices like the Union government, which typically buys much higher quantities of drugs. "We had to scramble to put together funds and procure drugs from manufacturers we haven't worked with before," says a TB officer from Tamil Nadu.

The Union health ministry has in multiple statements denied any shortage of TB drugs in India. However, no details of the stocks of the drugs that the states were asked to procure have been made available. TB advocates claim they continue to get distress calls from patients. "When a patient calls us to complain about the unavailability of a required dose or drug, we reach out to the state TB officers for assistance. They are usually very helpful. But a stock-out is beyond their control as well," says Ashna Ashesh, a public health advocate working with both the WHO and Central TB Division. An MDR-TB survivor herself, she says it is very important to closely monitor second-line drugs so that resistance doesn't set in. "We need to shift to community-level monitoring," she adds, "where community members can keep an eye on symptoms, drug shortages and other issues."

Prime Minister Narendra Modi had in March 2018 committed to eliminating TB from India by 2025, while other countries hoped to achieve TB-related Sustainable Development Goals by 2030. Letting the guard down will mean more than just failing to achieve a goal. We could well be staring at an outbreak of extremely drug-resistant TB. ■